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Five slides on what breaks in UK care data. Five on how HealthBridge is built to close it — structure taken from the working codebase.

FHIR R4 · SNOMED CT · NHS number · 10 agents

Independent project · no patient data · not affiliated with the NHS

THE PROBLEM · 1 OF 5

Four records, one patient, no join

GP practice

Independent contractors. The lifelong record sits in EMIS Web or SystmOne.

Hospital trust

Its own electronic patient record — separate system, separate login.

Pharmacy

Its own dispensing record, usually unconnected to the practice.

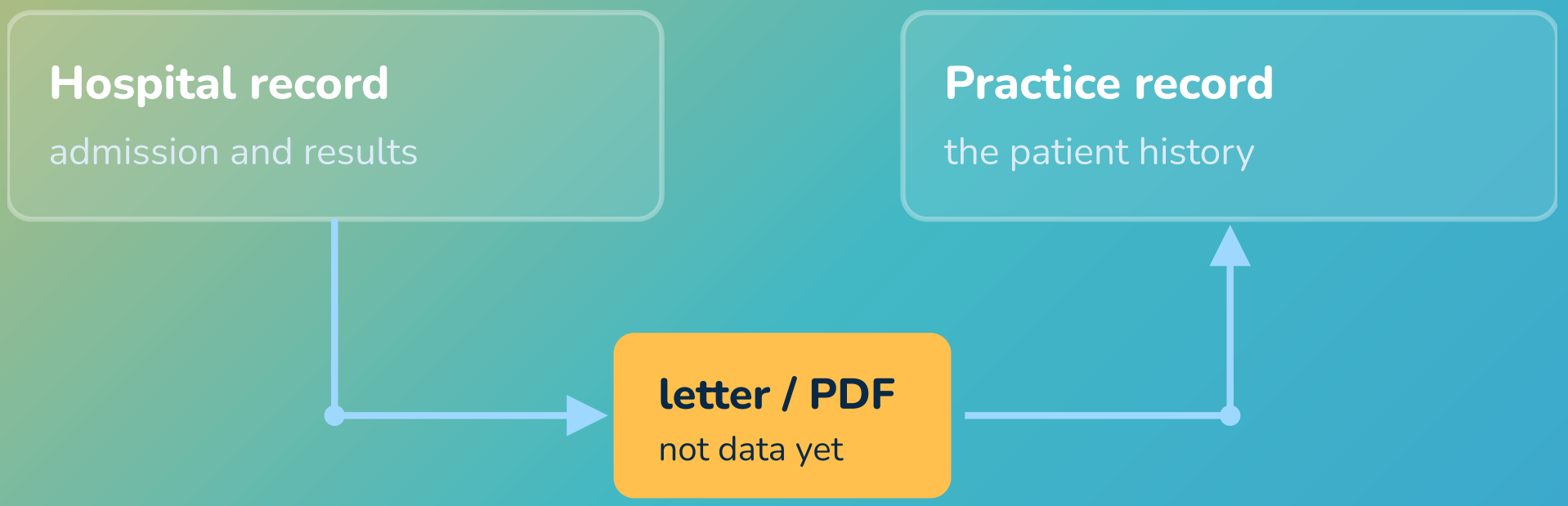
National

NHS number, Spine, GP Connect — plumbing, not the record itself.

So there is no single patient record. There are several.

THE PROBLEM · 2 OF 5

The join is a letter

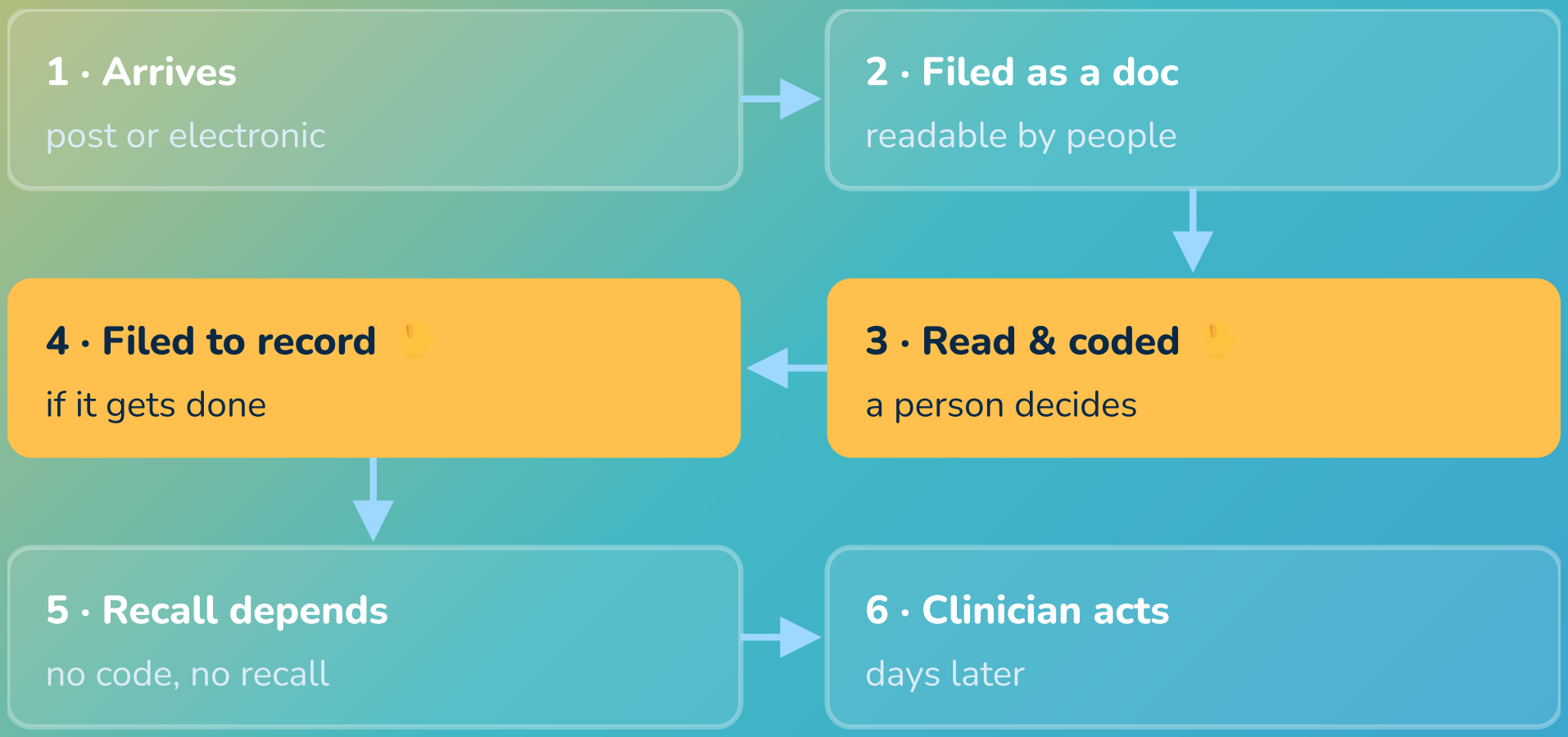


A person must read it and code it.

Until then the record does not know.

THE PROBLEM · 3 OF 5

Six hands, two of them manual



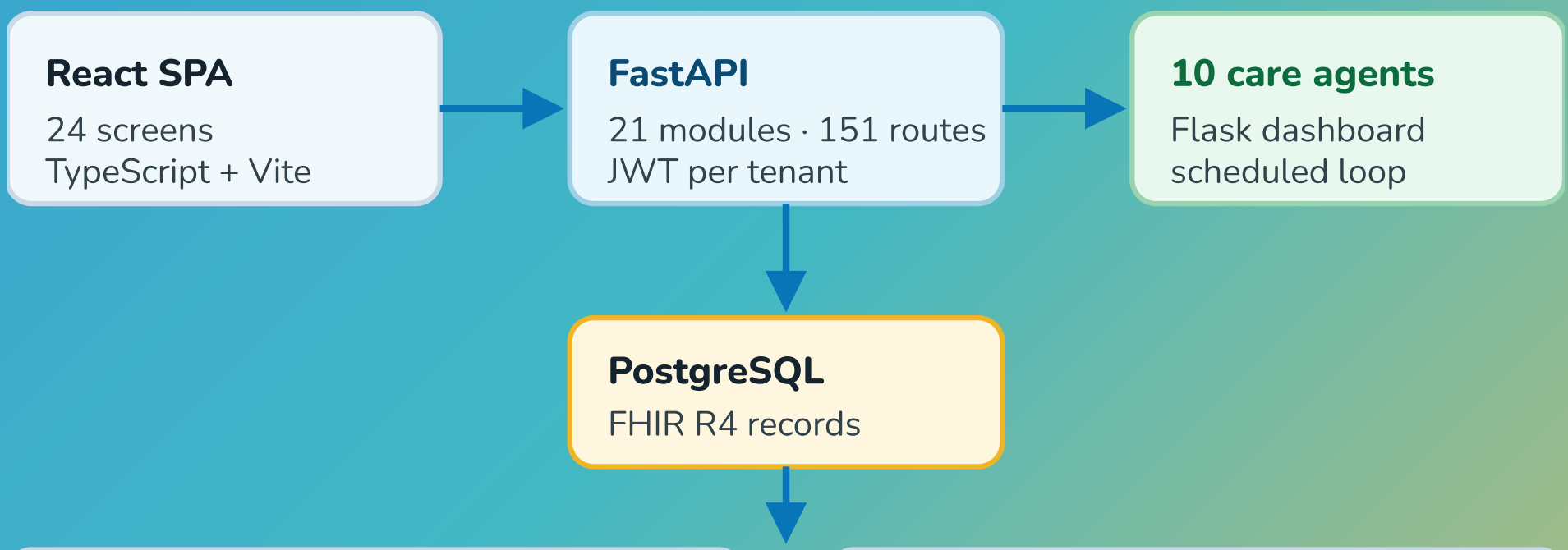
Steps 3 and 4 are where meaning is lost.

Four routine frictions

- **Coding lag** — the fact is on paper, not in the record being searched.
- **Recall misses** — a recall queries codes; free text is invisible to it.
- **Duplication** — the same datum typed again into a second system.
- **Delay** — a result in an inbox behaves like one that never arrived.

Then five slides on the build that closes the gap.

Where HealthBridge sits



One database, one record per patient.

One source of truth: agents read the same records clinicians write.

Tenant-scoped: the JWT carries the practice, every query is filtered.

FHIR out: records can leave as data, not as documents.

THE SOLUTION · 2 OF 5 · MAPPING

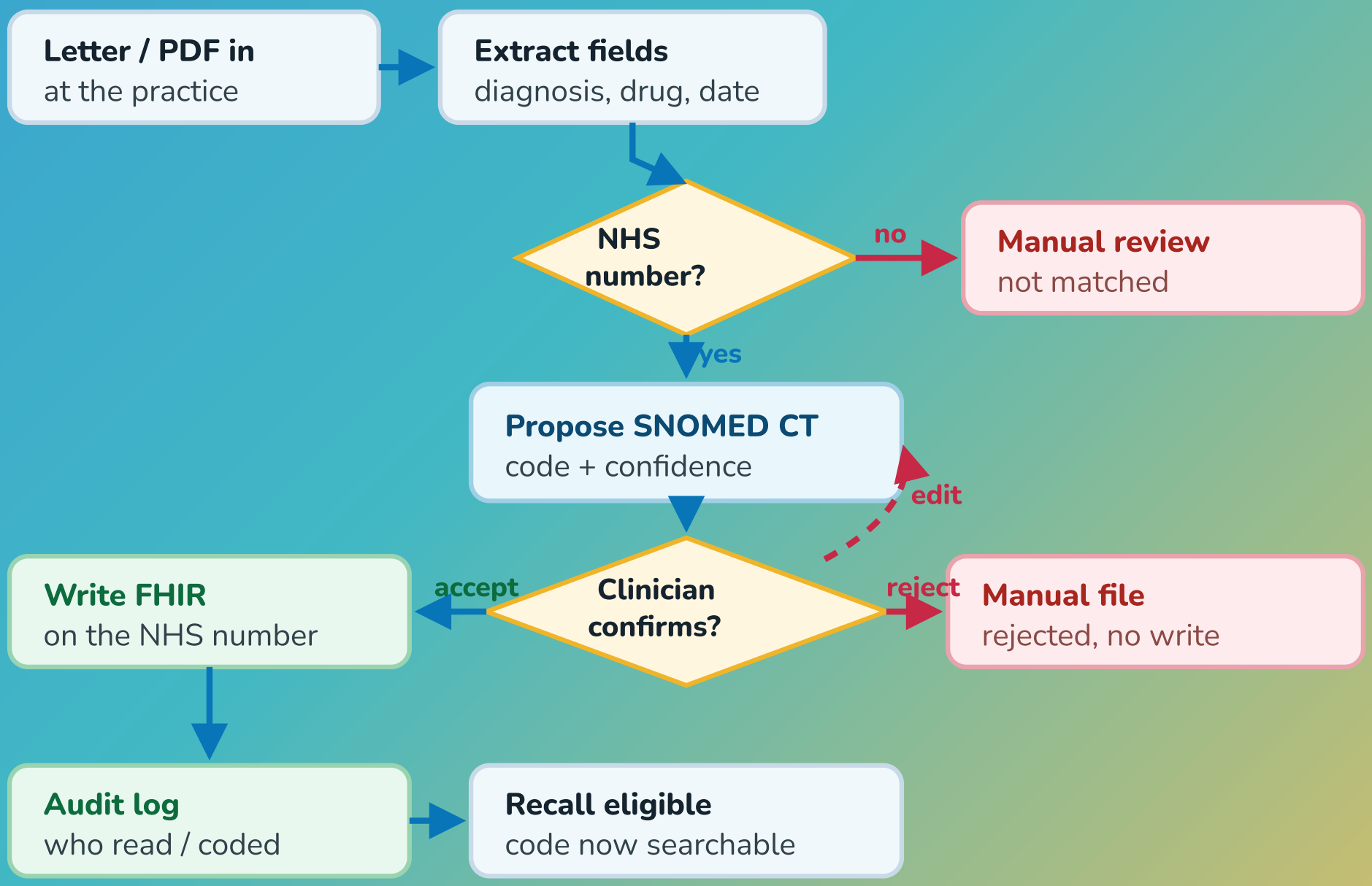
Built for India, mapped for the UK

IN THE CODE TODAY	UK EQUIVALENT
ABHA / UHID	NHS number
ICD-10 coding	SNOMED CT (SCCI0034)
Base FHIR R4	FHIR UK Core profiles
DPDP 2025 controls	UK GDPR + DPIA
WhatsApp bridge	SMS / NHS App messaging

The engineering travels: tenant isolation, audit trail, human confirmation. The terminology and the certifications do not — those are the work of a UK deployment.

THE SOLUTION · 3 OF 5 · FLOWCHART

What happens to one letter



THE SOLUTION · 4 OF 5 · IN THE REPO

What is actually built

151
REST endpoints across 21 modules

24
clinical screens, React + TypeScript

10
care agents on a scheduled loop

6
services: ingestion, FHIR, consent, messaging, identity, eKYC

Agents: patient intake · appointments · medication adherence · follow-up · labs · pharmacy · insurance · risk prediction · family care · voice care

Build loop reports **33 of 45 tasks** complete — foundation, compliance and the doctor workflow first.

THE SOLUTION · 5 OF 5

Three refusals, one honest limit

No diagnosis

No autonomous clinical decision, ever.

No shared pool

No cross-practice patient data.

No silent writes

Every filed fact is attributable.

No claims

No UK certification held today.

See the loop in 30 seconds — six clickable screens, sample data.

19p1362.github.io/portfolio/healthbridge

DTAC, DSPT, DPIA and DCB0129 are what a UK deployment would require first. Gonela Abhilash · CPC · registered pharmacist · MSc Public Health, ARU.